

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: TMiller **Period:** July 3 to October 1, 2024 **Report Date:** October 2, 2024 **Coverage:** 1608/2184h (73.6%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 91 days from Jul 03 to Oct 01 ■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
70	108h	Low Heart Rate	<1	74%

Major Findings: Sustained low HR (avg 50, min 40)

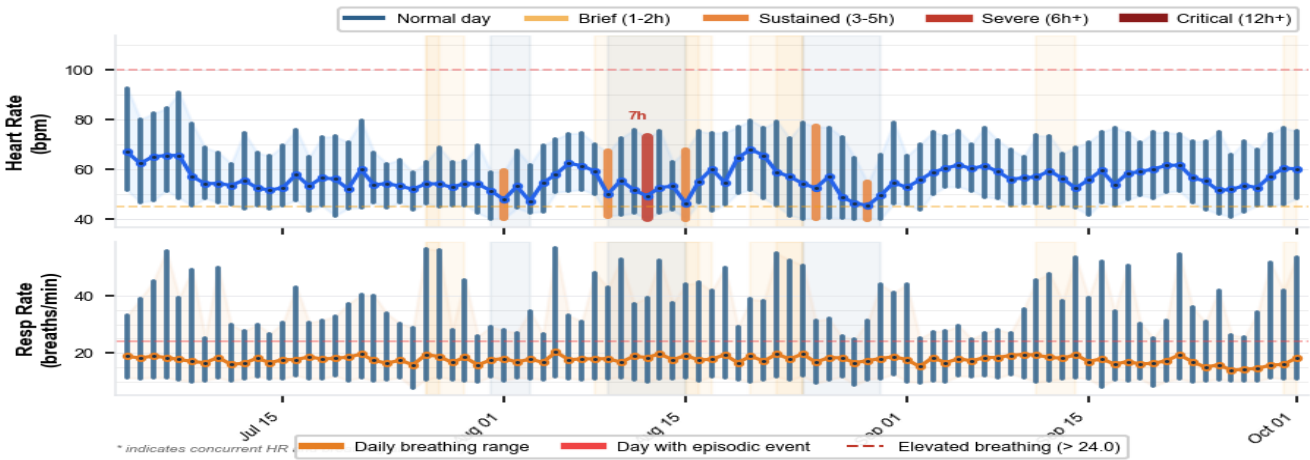
#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Low Heart Rate	40 bpm	24h	7h	1	51 bpm	Aug 09 to Aug 15
2	Low Heart Rate	40 bpm	28h	5h	2	50 bpm	Aug 24 to Aug 30
3	Low Heart Rate	40 bpm	8h	4h	<1	50 bpm	Jul 31 to Aug 03
4	High Breathing	56 brpm	3h	1h	2	19 brpm	Jul 26 to Jul 27
5	High Breathing	45 brpm	2h	1h	1	18 brpm	Aug 15 to Aug 16
6	High Breathing	55 brpm	2h	1h	<1	19 brpm	Aug 22 to Aug 24

+ 5 additional condition(s) across Elevated Breathing; details in monitoring data.

The P5 to P95 heart rate spread was 23 bpm (46 to 69), indicating significant cardiac variability.

Clinical Pattern Observations:

- 1. **Sustained finding:** 7h continuous Low Heart Rate on Aug 12.
- 2. **High variability:** HR spread 23 bpm (45 to 68).
- 3. **Clustered pattern:** Episodic events on 6 of 91 days (6%).



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.